



OFFICIAL

Identification of Patients in the Armadale Mental Health Service Procedure

1. Applicability

Site	Operational Area	Applicable to
Armadale Mental Health Service	Mental Health	Medical, Nursing and Allied Health

2. Purpose

The purpose of this procedure is to ensure accurate identification of patients, resulting in the administration of intended clinical interventions.

This procedure should be read in conjunction with the East Metropolitan Health Service (EMHS) [Patient Identification and Procedure Matching Policy EMHS: 21.](#)

It is a requirement that **all** patients be positively identified prior to patient registration / admission and throughout their treatment in any area of the Armadale Mental Health Service (AMHS).

Three identifiers are the approved minimum level of patient identification that must be confirmed in correctly identifying all mental health patients whenever care is provided. These three primary identifiers are:

- Name – Family and Given names.
- Date of birth.

Unique Medical Record Number (UMRN).

3. Procedure

- A patient must provide their full family and given names, date of birth, address and known allergies.
 - Where a patient is unable to provide this information, the legal guardian / next of kin / carer may identify the patient.
 - This is to be documented in the patient's health record.
- Wherever possible a Medicare card and / or other documented identification will be provided by the patient / carer / legal guardian.
- Staff are to consider cultural naming conventions and the use of preferred names rather than correct names when determining identification information.
 - Aliases are to be added to webPAS for patients who use alternate names.

Before referencing this procedural document please ensure that you have the latest version from the [Policy Library](#) information hub page.

- The three primary identifiers **must** be used when providing care, therapy or services. This includes:
 - On admission or activation to AMHS for inpatient, Crisis Resolution Home Treatment Team (hospital in the home type) service, community services and community residential.
 - During any provision of care, therapy or services, including medication administration, and consultation liaison.
 - Refer to [Medication Management Procedure](#)
 - During patient handover internally between areas of AMHS, transfer to external care providers or upon discharge from AMHS into the community.
 - Refer to the [WA Health Clinical Handover Policy](#)
- Incidents relating to matters of incorrect identification, procedure matching, or where a clinical incident results, must be reported using the clinical incident reporting system (Datix CIMS) and managed in accordance with the [WA Health Clinical Incident Management Policy](#)

3.1 Inpatients

- All inpatients with non-known allergies will be provided with a white identification band.
 - A patient label containing all three primary identifiers will be placed on the insert and then into the band window.
 - There may be situations where the wearing of a patient identification band is inappropriate, due to a patient's diagnosis, behavioural presentation and any risk to skin integrity.
 - In this instance, the reasons for not wearing an ID band must be documented in the patient's health record with the patient's preferred method of identification considered and the process for identification defined.

3.2 Photographic Identification

- As an additional means of identification, photographic identification is to be used for all inpatients with consent.
 - Refer to the [WA Health Consent to Treatment Policy OD 0657/16.](#)
- The photograph will be used solely for the period of the current admission and identification of the patient.
- The rationale for use of photographic identification is to be explained to the patient with reassurance that confidentiality will be maintained and the photograph disposed, by way of the blue confidential document disposal bin, as part of their discharge process.
- A photograph using an instant camera, of the head and shoulders of the patient will be taken by the admitting nurse and attached to the patient's medication chart as soon as practicable, with the reason for any delay clearly indicated in the health record and admission checklist.
 - Formal consent (verbal or written) to photograph involuntary patients must be obtained from the patient, or patient's next of kin / carer / legal guardian and documented in the health record.
 - The photograph must be labelled with the correct patient addressograph label and dated.
 - If this is not available, the patient's full name, date of birth and UMRN must be written on the back of the photograph and signed and dated by the staff member taking the photograph.

- Where the patient photograph may need to be transferred to a new medication chart or separated from the health record at any time, staff must confirm and match the information on the photograph to the health record, before placing the photograph into the health record.
- This must be done using the three primary identifiers which may include the patient confirmation of details.
- If the patient refuses ID band / and or photography, staff are to revert to the three primary identifiers:
 - Name – Family and Given names.
 - Date of birth.
 - Unique Medical Record Number (UMRN).

3.3 Clinical Alerts

3.3.1 Known Allergies / Adverse Drug Reaction

- Where a patient has a known allergy or has ever had an adverse reaction, including but not exclusive to, drugs, food, lotions and latex, this will be documented throughout the health record and medication chart and recorded in the patient administration system.
 - Refer to the [WA Health Patient Alert Med Alert Policy](#)
- A red ID band will be used in place of the white bracelet and will also have a patient addressograph label attached.
- The details of the allergy or reaction are not to be documented on the ID band but are to be entered into the health record.
- Refer to the [WA Health Patient Alert Med Alert Policy](#) for clinically important risks requiring documentation and notification, including the issuing of red patient ID bands.
- In the case of a previous adverse drug reaction, staff will follow the [Adverse Drug Reaction and Clinical Alert Procedure](#)

3.4 Same / Similar Names

- Patients with the same / similar names are to have an MR 620 cautionary label - 'Caution, patient with same or similar name. Check', placed on the front of the health record, on medication charts, consent forms and all other instructive documentation.
 - Features / markings, e.g. tattoos and birthmarks may also provide a secondary visual identifier and must be documented on the Physical Appearance Form (TMHMR904) and filed in the health record.

3.5 Unidentified Patients

- When a patient's identity cannot be reliably confirmed, e.g. the patient is unconscious, intoxicated or due to a compromised mental state, they must be registered as an 'Unknown Male' or 'Unknown Female' and an emergency UMRN is to be used.
 - The addressograph will be placed on a white identification band which will remain on the patient until discharge, or until identification has been determined and the details corrected

3.6 Outpatients – Community Clinics / Community Rehabilitation

- In non-inpatient areas where the identity of a patient needs to be checked and the patient is not wearing a name band, the patient's name, address, date of birth and other collateral information, must be checked against the health care record identification label.

As per the medication management procedure, patient identification to be checked prior to administering medication.

3.7 Photographic Identification

- As an additional means of identification, photographic identification is to be considered for community patients with consent, the photograph being used solely for the period of the current admission and identification of the patient. Refer to the [WA Health Consent to Treatment Policy](#).
- The rationale for use of photographic identification is to be explained to the patient with reassurance that confidentiality will be maintained and the photograph disposed, by way of the blue confidential document disposal bin, as part of their discharge process.
- A photograph using an instant camera, of the head and shoulders of the patient, will be taken by the care coordinator and attached to the patient's health record on the top left side of the inside front cover. If the patient is prescribed a depot medication, a photograph should be attached to the depot chart.
 - This is to occur as soon as practicable, with the reason for any delay clearly indicated in the health record and admission checklist.
- Formal consent, (verbal or written), to photograph involuntary patients (patients under a Community Treatment Order) must be obtained from the patient, or patient's next of kin / carer / legal guardian and documented in the health record.
 - The photograph will need to be transferred to a new health record volume if created.
- The photograph must be labelled with the correct patient addressograph label and dated.
 - If this is not available, the patient's full name, date of birth and UMRN must be written on the back of the photograph and signed and dated by the staff member taking the photograph.

4. Compliance monitoring

Compliance against this procedure will be monitored by the Safety and Quality – Mental Health. Compliance monitoring activities relevant to this procedure include:

- Audit of patient identification processes
- Monitoring of reported clinical incident data via Datix CIMS.

Specific findings are to be reported to the relevant departmental meeting for review and identification of any opportunities for improvement.

5. Policy context

The following documents are required to give effect to this procedure:

- Legislation
 - WA Mental Health Act 2014
- HDWA
 - [WA Health Patient Alert Med Alert Policy](#)

- [Clinical Handover Policy](#)
- [Clinical Incident Management Policy](#)
- [Consent to Treatment Policy](#)
- EMHS
 - [Patient Identification and Procedure Matching Policy EMHS: 21](#)
 - [Clinical Photography and Videography EMHS: 81](#)
- AKG
 - [Adverse Drug Reaction and Clinical Alert Procedure](#)
 - [Medication Management Procedure](#)

6. Relevant standards

- National Safety and Quality Health Service (NSQHS) Standards (second edition):
 - Standard 1 – Action 1.11
 - Standard 2 – Action 4.7
 - Standard 6 – Action 6.5
- National Standards for Mental Health Services (2010):
 - Standard 2 – Criteria 2.4

7. Bibliography

1. [Australian Commission for Safety and Quality in Health Care Fact Sheet: Specifications for a standard patient identification band](#)
2. [Australian Commission for Safety and Quality in Health Care Factsheet, Patient identification and procedure matching Standard 5](#)

8. Glossary

The following definitions are relevant to this policy.

Term	Definition
Patient Identification	The purpose of verifying a patient's identity for the purpose of intervention.
Patient Identifiers	These are descriptors that when combined form a system of unique identification for each patient.

9. Document contact

Service Director - Mental Health

Enquiries relating to this procedure may be directed to:

Title: Nurse Unit Manager
 Department: Mental Health
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10. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V6	19 01 2026	19 01 2029	Full review, Broken links updated

11. Authorisation

This procedure has been authorised and issued by:

Authorised by	Dene Olive – A/ Service Director Mental Health
Authorisation date	16 01 2026
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